A cycle of fear and anxiety is rippling through many cancer patients in India. For 57-year-old Kaashvi Rao, a stage I ovarian cancer patient from Hyderabad, a routine chemotherapy session turned into a frantic search. Visiting her in-laws in Delhi, she had planned to receive her next cycle at Sir Ganga Ram Hospital. But the drug she needed was unavailable. Though advised to switch medicines, Rao chose to fly back to Hyderabad to continue treatment with the prescribed drug. Her experience reflects a shortage of two of India's most widely used chemotherapy drugs—cisplatin and carboplatin. "India's cancer centres are grappling with serious shortages of the most widely used chemotherapy medicines. This is not limited to one or two hospitals," says Dr Bhawana S. Awasthy, director, Medical Oncology, CK Birla Hospital, Gurugram.

The two platinum-based medicines are used to treat several cancers, including ovarian, cervical, lung, bladder, head and neck and gastrointestinal. They work by damaging the DNA of cancer cells, preventing them from multiplying. Platinum is the key active ingredient that enables these drugs to bind to cancer-cell DNA. A recent surge in global platinum prices, leading to falling exports from key producers like South Africa, coupled with supply-chain disruptions due to the Middle East war, has had a direct impact on manufacturing costs.

The shortage first surfaced in May and worsened through June as soaring prices and government price caps squeezed manufacturers, forcing them to cease production. Hospitals, particularly government-run facilities, have reported dwindling stocks, forcing patients to hunt across pharmacies. Oncologists say that although substitutes exist, platinum-based drugs are essential to treat some cancers.

Now, the Centre has moved in to improve supplies. In June, the National Pharmaceutical Pricing Authority (NPPA) approved a one-time 50 per cent increase in the ceiling prices of cisplatin and carboplatin after invoking special provisions under the Drug Price Control Order 2013. The temporary revision was cleared after manufacturers argued that soaring platinum prices had made production commercially unviable. Noting with concern the shortage, the NPPA said their "uninterrupted availability is critical for public health". The revised prices came into effect on June 11 and will be reviewed after six months. Dr Awasthy, however, cautions, "The government has approved price hikes to encourage manufacturers to resume production, but the impact will take time to filter through."



> **"All chemotherapy drugs are part of highly researched treatment protocols where leaving out even one component can reduce efficacy. Platinum-based drugs are widely used and a shortage can affect treatment across a wide spectrum of cancers"**
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> **DR DEEPAK JHA,** *Senior Consultant, Surgical Oncology, Artemis Hospital, Gurugram*

DOCTORS' DILEMMA

The shortage has forced oncologists to do a balancing act. Every patient without access to the two medicines is reviewed to determine whether another regimen can offer comparable outcomes or whether treatment should be deferred until the preferred medicine is available. Hospitals are coordinating with distributors, other centres and alternative procurement channels. "When a chemotherapy drug is unavailable, oncologists review national/ international guidelines, assess alternatives and discuss options with the patient," says Dr Anil Thakwani, director, Radiation Oncology, Sharda Care Healthcity, Greater Noida. "If no acceptable substitute exists, treatment may be rescheduled briefly or the patient referred to a centre where the drug is available. The priority is to aim for the best chance of cure or disease control."

Doctors say the consequences of a shortage are not uniform. Whether a temporary change in chemotherapy affects treatment depends first on the type of cancer being treated and how strongly that particular tumour is treated by platinum-based drugs. For example, in testicular cancer, where cure rates are among the highest, and in several bladder, cervical and head and neck cancers, platinum-based drugs—particularly cisplatin—are the cornerstone of treatment, making substitutions challenging.

Again, patients receiving chemotherapy with the aim of curing their cancer are managed differently from those receiving treatment to control advanced disease or improve quality of life. "In highly curable cancers, we have to be careful and rely on scientific evidence while choosing an alternative," says Dr Vishwanath S., senior consultant, Medical Oncology, Apollo Hospitals, Bengaluru. In advanced cancers where the goal is disease control, an alternative regimen may sometimes have far less impact on overall outcomes.

Doctors point out that modern chemotherapy protocols are backed by decades of clinical research. "All chemotherapy drugs are part of highly researched treatment protocols where leaving out even one component can reduce the efficacy of the regimen," says Dr Deepak Jha, senior consultant, Surgical Oncology, Artemis Hospital, Gurugram. "Drugs like platinum